

General principles of treatment

Ten new clinical trials on scabies treatment have been published since the previous guideline in 2010. Recent data focus on mass population treatment, usually with ivermectin. Based on comparative studies evaluating the efficacy of different antiscabetic treatments, a distinction is made between 'recommended' and 'alternative' treatments. The availability of antiscabetic drugs varies across European countries, leading to variability in treatment choices in practice.

Topical treatment should be applied to all skin areas, including the scalp, groin, navel, external genitalia, finger and toe web spaces, and beneath the nails. It should be applied at night and left in place for 8–12 hours. The skin should be cool and dry before application. A second application is recommended after 7–14 days. After treatment, patients should change into clean clothing. All close personal contacts of the patient should be treated simultaneously to prevent re-infestation.

Clothing, bedding, towels, and other personal items should be machine washed at 50°C or higher, dry-cleaned, or sealed in a plastic bag for 1 week.

Patients should receive a detailed explanation of their infestation along with clear written information.

The infestation is considered cleared if, 1 week after completion of treatment, there are no signs of active scabies (i.e., no new lesions or nocturnal pruritus). Post-treatment itching may persist for 2–4 weeks.

Recommended treatments

- **Permethrin 5% cream:** Apply from head to toe and wash off after 8–12 hours. Repeat treatment after 7–14 days {evidence level Ib; grade A recommendation}.

- **Oral ivermectin:** 200 micrograms/kg, taken with food, given as two doses 1 week apart {evidence level Ib; grade A recommendation}.
- **Benzyl benzoate 10–25% lotion:** Apply once daily at night for 2 consecutive days, with re-application after 7 days {evidence level IV; grade C recommendation}.

Alternative treatments

- **Malathion 0.5% aqueous lotion** {evidence level IV; grade C recommendation}.
- **Ivermectin 1% lotion:** Reported to be as effective as permethrin 5% cream {evidence level Ib; grade A recommendation}.
- **Sulphur 6–33%** (as cream, ointment, or lotion): One of the oldest antiscabietic agents. Effective with application on three successive days {evidence level Ib; grade A recommendation}.
- **Synergized pyrethrins:** Available as a foam in some countries and shown to be as effective as permethrin 5% cream {evidence level IIa; grade B recommendation}.
- **Lindane:** No longer recommended due to risk of neurotoxicity.

Crusted scabies

- **Topical scabicide** (permethrin 5% cream or benzyl benzoate 25% lotion): Apply daily for 7 days, then twice weekly until cure, **and**
- **Oral ivermectin:** 200 micrograms/kg on days 1, 2, and 8.